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PART 1

Standard Operating Procedures

These ten SOP sections define the foundational policies, governance structure, staffing requirements, clinical framework, and compliance obligations that govern every aspect of program operation. Every staff member is responsible for knowing and adhering to these policies.

1. Agency Overview & Governance

Reference: 10A NCAC 27G .0100; NC Medicaid Managed Care Tailored Plan Requirements

1.1 Mission Statement. Well Spring Intervention LLC is dedicated to providing trauma-informed, high-quality supervised living environments for youth with severe emotional disturbances (SED), ensuring their safety and clinical growth. Our program philosophy is rooted in the belief that every child deserves a stable, predictable, and nurturing environment in which healing can occur. All staff actions, decisions, and interventions must align with this mission and reflect our core values of safety, dignity, accountability, and growth.

1.2 Licensing & Credentialing. The facility operates under a valid NC DHSR license (Level 3 Supervised Residential Group Home) and is credentialed as an In-Network Provider with the regional LME/MCO/Tailored Plan. The Executive Director maintains the original license on site, posts a current copy in a public area of the facility, and renews it prior to expiration. Any change in ownership, capacity, population served, or physical location requires prior written approval from DHSR and notification to the LME/MCO.

1.3 Corporate Compliance. Well Spring Intervention LLC maintains a Corporate Compliance Plan to prevent Medicaid fraud, waste, and abuse. All staff receive compliance training upon hire and annually thereafter. The plan designates a Compliance Officer (the Executive Director or designee) who receives reports, investigates concerns, and coordinates with the LME/MCO and Medicaid Fraud Investigations Unit when warranted. Retaliation against any person reporting a good faith compliance concern is strictly prohibited and is itself a terminable offense.

1.4 Organizational Structure. The Executive Director holds overall authority and accountability for facility operations. A Qualified Professional (QP) serves as Clinical Director and is responsible for clinical services, person-centered planning, and supervising Associated Professionals (APs) and Direct Care Professionals. An on-call QP rotation ensures 24/7/365 access to clinical decision-making authority. All staff report incidents, concerns, and operational needs through the documented chain of command.

2. Human Resources & Staffing Requirements

Reference: 10A NCAC 27G .0203 & .5600; NC Medicaid CCP 8C

2.1 Staffing Ratios. Minimum staffing ratios are mandated at all times to ensure resident safety and adequate supervision. Day and evening shifts maintain a 1:4 staff-to-resident ratio (awake and on-site), while the overnight shift maintains a 1:8 ratio with staff awake at all times. A Qualified Professional (QP) is on-call 24/7/365 and reachable within 30 minutes for emergencies. Ratios may be increased based on acuity, behavioral incidents, or as directed in a resident's Person-Centered Plan (PCP) — they may never be decreased below these minimums.

Shift	Minimum Ratio	Status	Notes
Day (7a–3p)	1 : 4	Awake / On-site	May increase per PCP acuity

Shift	Minimum Ratio	Status	Notes
Evening (3p–11p)	1 : 4	Awake / On-site	May increase per PCP acuity
Night (11p–7a)	1 : 8	Awake / On-site	15-min room checks required
On-Call QP	1 : —	Reachable ≤30 min	24/7/365 coverage

2.2 Staff Qualifications. All staff meet minimum credentialing requirements per 10A NCAC 27G .0203 prior to independent duty. QPs hold a Master's or Bachelor's degree in a human services field with the required supervised experience and are licensed or licensed-eligible in their discipline. APs hold a Bachelor's in a human services field with at least one year of relevant experience. Direct Care Professionals (PPs) hold a high school diploma or GED with at least one year of mental health experience. Personnel files document credentials, references, training, and ongoing supervision.

2.3 Background Checks. Prior to any unsupervised contact with residents, every employee and contracted staff member must clear: (a) NC SBI fingerprint criminal background check; (b) Health Care Personnel Registry check; (c) DSS Child Abuse and Neglect Registry check for every state of residence in the prior five years; and (d) Motor Vehicle Record (MVR) check for any staff who transport residents. Disqualifying findings follow 10A NCAC 27G .0204 and the agency's Criminal Background Check Policy. Re-checks are completed annually and upon reasonable suspicion.

2.4 Mandatory Training. All staff complete the following training prior to independent duty, with refreshers as indicated: CPR / First Aid (annually); NCI or CPI restraint and de-escalation (annually); Medication Administration (RN-delegated, annually); Bloodborne Pathogens (annually); Trauma-Informed Care (annually); and Rule 108 incident reporting (annually). Additional topics — adolescent development, suicide risk assessment (C-SSRS), and person-centered planning — are completed during orientation and reinforced through monthly staff meetings and quarterly in-services.

3. Admissions, Discharges, and Transition Planning

Reference: 10A NCAC 27G .5604; NC Medicaid CCP 8C

3.1 Admission Criteria. The program serves youth with a primary mental health or behavioral diagnosis requiring supervised living, who are medically stable, and whose clinical needs can be safely met in a Level 3 group home setting. The QP reviews each referral packet — including clinical assessment, current medications, IEP if applicable, and any history of aggression, elopement, or self-harm — to confirm appropriateness before admission is scheduled. Youth with active psychosis requiring IVC, medical instability, or recent fire-setting that cannot be safely managed in a group home are not appropriate for admission.

3.2 Admission Process. On the day of admission, the QP obtains signed consents for treatment, medication administration, transportation, release of information, and photographs. The Direct Care Professional (DCP) conducts a respectful property search (see Protocol 9), secures all medications in the double-locked med cart, and orients the youth to the facility, schedule, and house rules. The QP administers the C-SSRS suicide screening and documents the youth's immediate clinical status. An RN visit is scheduled within 72 hours for medication delegation and a nursing assessment.

3.3 Discharge & Transition. A transition plan is initiated at admission and reviewed every 30 days during the PCP cycle. Discharge may be planned (return to family, step-down to a less restrictive setting, or aging out) or unplanned (acute hospitalization, IVC, or placement disruption requiring higher level of care). A discharge summary, including clinical progress, current medications, follow-up appointments, and recommendations, is completed within seven

calendar days of discharge and provided to the guardian, receiving provider, and LME/MCO.

4. Clinical Services & Person-Centered Planning

Reference: 10A NCAC 27E .0300; NC Medicaid CCP 8C

4.1 PCP Development. Within 30 calendar days of admission, the QP facilitates a Person-Centered Plan (PCP) meeting that includes the youth, guardian, LME/MCO representative, and any other supports identified by the family. The PCP documents the youth's strengths, needs, goals, and specific interventions, and is signed by all participants. The PCP is reviewed every 90 days thereafter, with addendums completed for any significant clinical change (new diagnosis, medication change, major incident, or change in living arrangement). Staff actively implement PCP goals during daily routines and document progress in shift notes.

4.2 Service Delivery. All services are delivered in accordance with the PCP. Staff implement goals daily through structured activities, behavioral interventions, skill-building, and clinical groups. Progress notes must reference the specific ISP goal being addressed and describe the youth's response objectively. The QP reviews progress notes weekly to ensure clinical fidelity and adjusts interventions as needed. Individual therapy, family therapy (when indicated), and psychiatric medication management are delivered by appropriately credentialed external or contract providers, with the QP coordinating all care.

4.3 Clinical Supervision. The QP provides documented clinical supervision to all APs and Direct Care Professionals at least monthly, with additional supervision following any critical incident, restraint, or significant behavioral change. Supervision notes document case review, skill development, and any performance concerns. The QP is responsible for ensuring that all direct care staff understand and can correctly implement each resident's Behavioral Support Plan (BSP) and Safety Plan.

5. Behavioral Management & Restraint

Reference: 10A NCAC 27G .0209; CMS Mental Health Parity Rules

5.1 Philosophy. Well Spring Intervention LLC employs a trauma-informed, positive behavioral interventions model. We recognize that behavior is communication, and our role is to teach and reinforce safer, more adaptive skills rather than punish symptoms of trauma or disability. Punitive measures — including corporal punishment, withholding of meals or sleep, social isolation as a consequence, and any intervention designed to humiliate or degrade — are strictly prohibited and constitute reportable abuse under Rule 108.

5.2 Behavioral Support Plan (BSP). A BSP is developed by the QP within 14 calendar days of admission, based on a Functional Behavioral Assessment (FBA) that identifies the antecedents, function, and consequences of target behaviors. The BSP specifies proactive strategies, replacement skills, de-escalation steps, and crisis responses tailored to the youth. All staff who work with the youth must review and sign the BSP Acknowledgment Form. The BSP is updated as needed based on data review during the 90-day PCP cycle.

5.3 Restraint. Physical restraint is the absolute last resort, used only when a youth is in imminent danger of serious harm to self or others, and only after less restrictive de-escalation attempts have failed. Mechanical restraints, chemical restraints, and seclusion are prohibited. Only NCI/CPI-trained staff may apply restraint, using the least restrictive hold that effectively prevents harm, and only for the minimum duration necessary. Prone (face-down) positions are never used. A post-restraint medical check, debriefing with the youth, and guardian notification are required for every restraint. All restraints are reported in IRIS.

6. Health, Medication, & Nutrition Management

Reference: 10A NCAC 27G .0209; NC Nursing Practice Act

6.1 Medical Care. Each resident has an identified Primary Care Physician (PCP) and psychiatrist upon admission. A complete medical examination is conducted within 30 days of admission and annually thereafter. The QP schedules and tracks all medical and psychiatric appointments; follow-up visits are scheduled as clinically indicated. A medical history, including immunizations, allergies, and current prescriptions, is maintained in the youth's chart and updated at every visit.

6.2 Medication Management. All medications are stored in a double-locked cabinet or cart, with controlled substances counted and documented at every shift change. Medications are administered only by RN-delegated staff who have completed the NC Medication Administration training. Staff follow the "5 Rights" (right youth, right medication, right dose, right route, right time) and document administration on the Medication Administration Record (MAR) immediately — never retrospectively. Medication errors, refusals, and adverse reactions are reported to the RN and QP immediately, documented on the MAR, and entered into IRIS as required.

6.3 Nutrition. Meals follow USDA guidelines for nutritional adequacy and are planned in collaboration with the youth when possible. Special diets — medical, religious, or cultural — are accommodated with documentation from the prescribing provider. Menus are posted in the kitchen and retained for 30 days. Food is never withheld as a behavioral consequence; snacks are available as part of the structured daily routine. All kitchen staff and direct care staff who prepare food maintain current food handler certifications.

7. Education & Vocational Support

Reference: NC General Statutes (Education of Homeless Children); IDEA

7.1 Education Coordination. The QP serves as the Education Liaison for every youth in the program. Within 10 calendar days of admission, the QP contacts the Local Education Agency (LEA) — specifically the McKinney-Vento liaison and the EC Director if the youth has an IEP — to coordinate enrollment, transportation, and service continuity. The QP signs a Release of Information (ROI) authorizing communication with the school, attends all IEP meetings, and tracks daily school attendance. Any school refusal is treated as a clinical issue and addressed through the PCP rather than as a behavioral violation.

7.2 Vocational Support. For youth age 14 and older, the PCP includes vocational and independent-living goals aligned with the youth's transition plan. The QP coordinates with Vocational Rehabilitation, community-based employment programs, and after-school job opportunities as appropriate. Direct care staff support skill development in meal preparation, laundry, budgeting, transportation, and self-advocacy through structured daily routines.

8. Incident Reporting & Response (Rule 108 / IRIS)

Reference: 10A NCAC 27T (Rule 108)

8.1 Reportable Incidents. The following events are reportable under Rule 108 and must be entered into the Incident Reporting and Investigative System (IRIS): death (including suicide attempts resulting in death), serious injury, alleged abuse or neglect, sexual abuse allegations, use of physical restraint, AWOL or missing child, medication errors, suicidal ideation or self-harm, and any other event meeting the regulatory definition of a reportable incident. When in doubt, staff report — the QP makes the final determination of reportability.

8.2 Reporting Timeframes. Staff must notify the On-Call QP immediately upon discovery of a reportable incident. The QP enters the IRIS report within 24 hours of discovery. Allegations of abuse, neglect, or a missing child trigger immediate (within 15 minutes) notification to DSS and law enforcement as appropriate. The guardian is notified within one hour of incident discovery unless doing so would compromise an active investigation. Verbal

notifications are followed by written documentation.

8.3 Investigations. An internal investigation is initiated within 24 hours of incident discovery and completed within 14 calendar days. The investigation includes interviews with all involved parties, review of video footage if available, documentation review, and a written narrative of findings. A Corrective Action Plan (CAP) is developed for any identified systemic or individual performance issue and uploaded to IRIS. All investigations are conducted objectively, with attention to preserving evidence and protecting the rights of all parties involved.

9. Facility, Safety, & Environmental Management

Reference: 10A NCAC 27G .0600; NC Fire Code

9.1 Environment. The facility maintains a safe, clean, and home-like environment. Maximum occupancy is two youth per bedroom, with each youth having individual storage space for personal belongings. Indoor temperatures are maintained between 68°F and 80°F. Hot water at fixtures used by youth does not exceed 120°F to prevent scald injuries. Bedrooms, bathrooms, and common areas are inspected daily by direct care staff and weekly by the QP for safety hazards; identified hazards are corrected immediately or documented and escalated.

9.2 Drills & Inspections. Monthly fire drills are conducted and timed — target evacuation time is under three minutes. Quarterly tornado drills are conducted using the designated interior safe room. Smoke detectors, fire extinguishers, sprinkler systems, and carbon monoxide detectors are inspected monthly by designated staff and annually by licensed contractors. Inspection records are maintained on the Environmental Safety Log (Form 5) for a minimum of three years.

9.3 Hazardous Materials. All cleaning chemicals, sharps, tools, and medications are stored in locked areas inaccessible to youth. The facility maintains Safety Data Sheets (SDS) for every chemical on site, organized in a binder in the kitchen and staff office. No weapons — including firearms, ammunition, knives beyond kitchen use, or any item modified for use as a weapon — are permitted on the grounds at any time, including in staff vehicles parked on the property.

10. Medicaid Billing & Documentation Compliance

Reference: NC Medicaid CCP 8C; CMS Documentation Guidelines

10.1 Progress Notes. Every staff member who provides direct care completes a shift note by the end of their shift. Notes must be objective, factual, and specific — avoiding labels and interpretations. Each note must reference the ISP goal(s) addressed, the intervention provided, and the youth's response. Notes are signed with the staff member's full name, credentials, and date. Backdating is strictly prohibited and constitutes Medicaid fraud. The QP audits shift notes weekly for compliance. The guiding principle is simple: *if it is not documented, it did not happen.*

10.2 Billing. The program bills Medicaid on a daily per diem basis for each eligible youth. Billing is suspended for any day the youth is hospitalized, detained, or on a home pass exceeding 24 consecutive hours. The Billing Coordinator reviews Medicaid eligibility monthly and notifies the QP immediately of any eligibility change. The Home Pass & Medicaid Billing Exclusion Tracker (Form 4) documents all absences and the corresponding billing action. Billing errors are corrected promptly and reported to the LME/MCO as required.

10.3 Record Retention. All clinical, medical, billing, and personnel records are retained for a minimum of 10 years from the date of discharge or termination, or longer if required by law or pending litigation. Records are stored in locked, climate-controlled areas with access limited to authorized personnel. Electronic records are backed up daily and stored securely off-site. Records are released only with a signed ROI or as required by law, court order, or regulatory audit.

PART 2

Actionable Workflows & Protocols

These 19 protocols translate the SOP policies into clear, sequential, step-by-step actions for daily operations. Each protocol is designed to be referenced quickly during a shift and followed exactly. Deviations from protocol require QP approval and must be documented.

1. Admissions Protocol

Pre-Admission. QP reviews referral packet, verifies active Medicaid, and schedules a tour with the guardian and youth.

Day of Admission. QP obtains all signed consents. DCP conducts a respectful property search and secures medications. QP administers the C-SSRS suicide screening.

Post-Admission. RN visits within 72 hours for medication delegation and nursing assessment. ISP/PCP meeting is scheduled within 30 days.

2. Daily Direct Care Protocol (DCP)

Day Shift (7a–3p). Shift change handoff and controlled-substance med count. Wake youth per schedule with 15-minute visual checks. Breakfast and AM medications. School transportation. Daily chores.

Evening Shift (3p–11p). Shift change. Therapy and clinical groups. Chores, dinner, and PM medications. Free time and hygiene routine. 10:00 PM bedtime.

Night Shift (11p–7a). Shift change and med count. Conduct 15-minute line-of-sight room checks documented on the Night Watch Log. Awake at all times — no sleeping permitted.

3. Incident Response & IRIS Protocol

Immediate. Ensure scene safety; call 911 if medical emergency or active danger. Notify the On-Call QP immediately.

Reporting. QP enters the IRIS report within 24 hours using objective, factual narrative — no interpretations or labels.

Notifications. DSS immediately for abuse allegations. Guardian within 1 hour. LME/MCO per their requirements.

Investigation. QP completes the internal investigation within 14 days and uploads findings and CAP to IRIS.

4. Behavioral Crisis & Restraint Protocol

Tier 1 — De-escalation. Use BSP coping skills: sensory items, breaks, verbal redirection, change of environment.

Tier 2 — Crisis. Clear the area of other youth. Call for staff backup. Use the NCI/CPI verbal continuum and physical distancing.

Tier 3 — Restraint. Only for imminent serious harm. Apply NCI hold; never use prone, mechanical, or chemical restraints. End the hold the moment danger passes.

Post-Restraint. Medical check within 1 hour. Notify guardian within 1 hour. Debrief with youth within 24 hours. File IRIS report.

5. Medication Administration Protocol

Prep. Wash hands. Unlock med cart. Verify the "5 Rights" against the MAR: right youth, right med, right dose, right route, right time.

Administer. Verify youth identity. Pour medication into a labeled cup. Observe swallowing. Check the mouth for "cheeking".

Document. Initial the MAR immediately. Circle "R" if refused; notify RN and QP. Never pre-initial or backdate.

Errors. Stop, isolate remaining meds, call RN/Psychiatrist, monitor the youth, file IRIS, and notify the guardian.

6. Medicaid Documentation & Shift Note Protocol

Write the shift note by end of shift. No backdating — ever.

Objective Data. "John threw a plastic cup when told to turn off the Xbox."

Action. "Staff utilized BSP Step 2: verbal redirection and a 10-minute break."

ISP Linkage. "Relates to ISP Goal #1: managing anger."

Sign and lock the note with credentials. Billing suspends for any day the youth is away for more than 24 consecutive hours.

7. AWOL / Missing Child Response

Discovery. Call the staff code for missing youth. Search the immediate area and check all cameras.

Notifications (within 15–30 minutes). Law enforcement, guardian, LME/MCO, and DSS (if applicable).

Documentation. File IRIS within 24 hours with a complete narrative.

Return. Conduct a medical check, contraband search, and trauma-informed debriefing. Notify all parties who were previously contacted.

8. Suicide Risk & Self-Harm Management

Screening. C-SSRS at admission and immediately whenever threats or self-harm occur.

Safety. Remove hazards (ligatures, sharps, cords). Implement 1:1 line-of-sight supervision.

Clinical. QP assesses within 1 hour. If imminent risk: call 911 or initiate IVC. If moderate risk: update the Safety Plan and increase supervision.

Documentation. File IRIS. Staff sign the updated Safety Plan and acknowledge the increased supervision level.

9. Contraband & Room Search Protocol

Definition. Contraband includes weapons, drugs, unauthorized medications, lighters, chargers with frayed cords, and any item prohibited by house rules.

Routine. Conducted at admission and on return from any pass. The youth must be present.

Probable Cause. Requires QP approval. Two staff must be present. Document the basis for the search.

Execution. Search systematically (one area at a time). Safe items are locked in the safe and returned at discharge. Illegal items are turned over to law enforcement.

10. PCP Implementation & Review

Development (30 Days). QP schedules the meeting with the team and drafts measurable, individualized goals.

Operationalize. QP creates a "Staff Cheat Sheet" summarizing each youth's goals, triggers, and interventions. Staff sign the PCP Acknowledgment Form.

90-Day Review. QP tracks expirations and schedules reviews. Updates progress, obtains signatures, and writes addendums for any significant change.

11. Transportation & Community Outing Safety

Driver Prep. Valid license, clean MVR on file. Complete the daily vehicle safety checklist. Confirm first-aid kit and Medicaid cards are present.

Transport. Conduct a headcount before leaving and upon return. If transporting one youth, staff sits in the back seat. Seatbelts on at all times. Never leave youth unattended in a vehicle.

Outings. Tied to an ISP goal. Assess behavioral stability (no restraint in the last 4 hours). Carry the agency cell phone and emergency contact list.

12. Grievance Process for Youth

Accessibility. Process is explained at admission. Blank forms and a locked grievance box are located in the common area. LME/MCO grievance numbers are posted.

Filing. Verbal or written. Staff assist immediately upon request. Place the grievance in the locked box within 1 hour.

Resolution. QP opens the box daily. Acknowledges within 24 hours. Provides written resolution within 7 days. Abuse allegations trigger an IRIS report and immediate DSS notification.

13. Staffing Ratio & Awake Overnight Protocols

Ratios. 1:4 day and evening. If a staff member calls out, the on-call system activates a replacement; the QP covers in-house if no replacement is available.

Awake Overnight. Sleeping is prohibited. 15-minute visual room checks are documented on the Night Watch Log. Hallway lights remain on. Engage nighttime wakers quietly and briefly — do not start conversations that escalate arousal.

14. Medical Emergencies & Acute Illness Response

Triage. Assess the situation. Call QP and/or RN. Err on the side of caution — when in doubt, transport.

Emergency. Call 911. Staff rides in the ambulance. QP calls in additional staff to maintain ratio at the facility.

Non-Emergency. Contact PCP for a same-day appointment. Take consent forms and Medicaid card.

Documentation. Guardian notified within 1 hour. IRIS filed. Billing suspended if the youth is hospitalized overnight.

15. Educational Coordination & School Reintegration

Enrollment (5 Days). QP contacts the LEA McKinney-Vento liaison or EC Director. Signs ROI. Coordinates transportation.

IEP. QP/AP attends IEP meetings. Advocates for an aligned BIP. Files the IEP in the youth's chart.

Attendance. Morning check before transport. Log daily. Refusal is an ISP issue, not a punishable offense. Collect daily behavior reports from the school.

16. Family Engagement, Visitation & Home Passes

Visitation. Based on ISP/court orders. Approved visitor list is maintained in the office. Staff search bags before visits. Document all visits.

Home Passes. QP and guardian must approve. Pass medications are provided in a locked box with administration instructions.

Billing Exclusion. If the youth is away for more than 24 consecutive hours, Medicaid per diem is suspended. Notify the Billing Coordinator immediately.

17. Discharge & Transition Protocol

Planning. Begins at admission. Trial home visits are conducted prior to discharge whenever possible.

Step-Down. Discharge meeting is scheduled. Community linkages (outpatient therapy, medication management, school) are established and confirmed before discharge.

Discharge Day. Medications are handed directly to the guardian with a signed transfer form. Belongings are inventoried. Discharge summary is completed within 7 days.

18. Infectious Disease & Bloodborne Pathogen Control

Universal Precautions. Treat all bodily fluids as infectious. Wear appropriate PPE (gloves, mask, eye protection) when exposure is possible.

Exposure. Wash the affected area with soap and water. Notify QP and RN. Go to Urgent Care within 2 hours. File IRIS. Offer post-exposure testing per protocol.

Outbreak. Two or more youth with identical symptoms = isolate, increase sanitation, and notify the County Health Department if the illness is reportable.

19. Emergency & Disaster Preparedness

Fire. Evacuate immediately. Headcount at the rally point. Call 911. Do not re-enter. Monthly drills required.

Tornado. Move to the interior safe room. Turtle position. Quarterly drills required.

System Failure. Distribute flashlights. If heat loss exceeds 4 hours or water loss exceeds 8 hours, initiate the Emergency Relocation Plan.

Lockdown. Secure doors and windows. Hide. Silence phones. Do not open for anyone but law enforcement. Annual drills required.

PART 3

Customized Forms & Logs

The following six forms and logs are the official documentation instruments for daily operations. Each form must be completed in ink or in the electronic record system, signed by the responsible staff member, and filed in the youth's chart or the facility's operational binder as indicated. Originals are retained per the record retention policy (10 years).

1. Shift Change & Awake Night Watch Log

Facility: Well Spring Intervention LLC **Date:** _____

Awake Overnight Room Checks (Every 15 Minutes)

Instructions: Initial each box to verify you visually saw the youth breathing and in their bed.

Time	Youth 1	Youth 2	Youth 3	Youth 4	Staff Initials
11:00 PM					
11:15 PM					
11:30 PM					
11:45 PM					
12:00 AM					
12:15 AM					
12:30 AM					
12:45 AM					
1:00 AM					
1:15 AM					
1:30 AM					
1:45 AM					
2:00 AM					
2:15 AM					
2:30 AM					
2:45 AM					
3:00 AM					
3:15 AM					
3:30 AM					
3:45 AM					
4:00 AM					
4:15 AM					
4:30 AM					
4:45 AM					

Time	Youth 1	Youth 2	Youth 3	Youth 4	Staff Initials
5:00 AM					
5:15 AM					
5:30 AM					
5:45 AM					
6:00 AM					
6:15 AM					
6:30 AM					
6:45 AM					

Shift Handoff Verification: Controlled substance count verified.

Off-Going Staff: _____ On-Coming Staff: _____

2. Contraband & Belongings Inventory

Youth Name: _____ **Date:** _____

Search Type: ☐ Admission ☐ Return from Pass ☐ Probable Cause (QP Approval: _____)

Item Description	Quantity	Brought In / Found	Disposition (Kept / Safe / Guardian)	Staff Initials

Youth Acknowledgment: Belongings inventoried in my presence.

Youth Signature: _____ Staff Signature: _____

3. Physical Restraint & Debriefing Checklist

Youth: _____ **Date:** _____ **Time Started:** _____ **Time Ended:** _____

Total Duration (Min): _____ **Technique:** _____

Pre-Restraint De-escalation Attempts:

☐ Verbal redirection ☐ Offered break ☐ Sensory item ☐ BSP coping skill ☐ Separation

Reason for Restraint (Imminent danger):

☐ Danger to Self ☐ Danger to Others ☐ Property destruction posing risk

Describe objectively: _____

Post-Restraint Medical Check (Within 1 hour):Youth checked for injuries, breathing normally: ☐ Yes ☐ No (Seek medical attention)

Staff Signature: _____

Notifications:On-Call QP Notified: ☐ Y ☐ N Time: _____ Guardian Notified: ☐ Y ☐ N Time: _____IRIS Report Filed: ☐ Y ☐ N IRIS ID #: _____**Post-Restraint Debriefing (Within 24 hours):**

Youth debriefed by: _____ Date/Time: _____

Trigger? _____

Youth alternative? _____

Staff alternative? _____

Youth Signature: _____ QP Signature: _____

4. Home Pass & Medicaid Billing Exclusion Tracker

Youth: _____ Month/Year: _____

Date Left	Time Left	Destination / Pass	Date Returned	Time Returned	Total Hours Away	Billing Action (Suspension Days)	Staff Initials

Billing Coordinator Sign-Off: Medicaid billing adjusted for suspension days.

Signature: _____ Date: _____

5. Emergency Drill & Environmental Safety Log

Facility: Well Spring Intervention LLC**Monthly Fire Drills (Under 3 minutes)**

Month	Date	Time of Day	# Youth	Evac Time	Staff	Signature
Jan						
Feb						
Mar						
Apr						
May						
Jun						
Jul						

Month	Date	Time of Day	# Youth	Evac Time	Staff	Signature
Aug						
Sep						
Oct						
Nov						
Dec						

Quarterly Tornado Drills

Quarter	Date	# Youth	Staff	Signature
Q1				
Q2				
Q3				
Q4				

Monthly Environmental Checks

Month	Smoke Detectors (Y/N)	Extinguisher (Y/N)	Hot Water ($\leq 120^{\circ}\text{F}$)	Fridge ($< 40^{\circ}\text{F}$)	Staff Initials
Jan					
Feb					
Mar					
Apr					
May					
Jun					
Jul					
Aug					
Sep					
Oct					
Nov					
Dec					

6. Employee SOP Acknowledgment

Employee Name: _____

Title: ☐ QP ☐ AP ☐ Direct Care Professional

By signing below, I acknowledge that I have received, read, and understand the SOP Manual for **Well Spring Intervention LLC**. I understand these policies are mandated by NC DHSR (10A NCAC 27G), NC Medicaid (CCP 8C), and Rule 108 (10A NCAC 27T). I agree to follow these protocols exactly. I understand failure to do so may result in disciplinary action, termination, or legal consequences regarding Medicaid fraud.

Employee Signature: _____ **Date:** _____

QP / Supervisor Signature: _____ **Date:** _____